

Environmental Scan Summary

*Existing Youth Behavioral Health Data Sources Across
Larimer County School Districts*

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This Environmental Scan documents what Estes Park, Poudre, and Thompson School Districts currently measure about student behavioral health, what the verified local data shows, and where the critical gaps are.

Why This Scan Matters

Every day across Estes Park, Poudre, and Thompson School Districts, students walk into classrooms carrying more than their backpacks. Some are anxious. Some are deeply sad. Some are managing things at home that no young person should manage alone. Most of them, because they are not acting out or causing trouble, stay invisible to the adults around them.

We know this is happening because the data tells us so. In 2023, 25.2 percent of Larimer County high school students felt so persistently sad or hopeless that they stopped doing normal activities. Nearly one in eight seriously considered suicide. These numbers improved from 2021, and that reflects real work happening in schools and the community. But the progress is uneven. Hispanic and Latino students are experiencing persistent sadness at a rate six points higher than their White peers. Transgender students are experiencing mental health crises at rates that far exceed any county average. And without a coordinated school-level data system, we cannot see these patterns clearly, track whether the work is reaching the students who need it most, or coordinate a meaningful response across three districts that are currently operating largely in isolation from one another.

Larimer County has invested seriously in behavioral health. In 2018, 61 percent of voters passed a 20-year sales tax that now generates \$24.5 million annually. The Longview Campus opened in December 2023. The adolescent crisis stabilization unit for youth ages 12 to 17 opened in 2025. \$17.7 million in grants has been awarded to 77 community organizations (Larimer County Behavioral Health Services, 2025). What connects none of that investment to the students most at risk is a coordinated school-level data system. That is what this project is building, starting with documenting what exists and what is missing.

Three findings define the current state.

- No district collects annual school-level data on persistent sadness, suicidal ideation, self-harm, or experiences of racism. These are the indicators most critical to behavioral health planning and they are absent from every district's own annual survey.
- The five active survey instruments across the three districts measure overlapping constructs in incompatible ways. Direct cross-district comparison is not currently possible.
- PSD's five-year Connections dataset demonstrates what school-level behavioral health measurement can look like when it is working. Its equity disaggregation reveals patterns that county-wide averages consistently hide.

The Local Data: What We Know About Larimer County Youth

The 2023 Healthy Kids Colorado Survey collected responses from 5,034 Larimer County high school students (Colorado School of Public Health, 2023). It is the most comprehensive source of youth behavioral health data available for this county and provides the baseline against which any shared metric would be measured.

Larimer County students do better than the state average on several indicators. Stress manageability is 63.3 percent here versus 58 percent statewide. Trusted adult access is 79.9 percent versus 75 percent statewide. The trend confirms the community's investments are working: persistent sadness fell from 34.6 percent in 2019 to 41.3 percent in 2021, then to 25.2 percent in 2023. Trusted adult access improved from 75.4 percent in 2019 to 79.9 percent in 2023 (Groves, Jorgensen, & Olsen, 2024). Something is working in Larimer County, and it is worth acknowledging that.

But the within-county disparities tell a different story. The overall figures look reasonable. The disaggregated figures reveal who is being left behind.

Table 1. 2023 HKCS Larimer County High School Results

Source: HKCS High School Region 2 Report (n = 5,034). Verified from the 169-table regional PDF with 95% confidence intervals, accessed under Data Use Agreement. HKCS table numbers given for traceability.

Indicator (Table)	Overall	95% CI	Key equity breakdown
Persistent sadness/hopelessness (T35)	25.2%	(23.0, 27.3)	Hispanic/Latino 29.6%; transgender 66.4%; female 31.3%
Seriously considered suicide (T36)	11.9%	(10.1, 13.8)	Transgender 50.0%; bisexual 28.2%; female 13.3%
Made a suicide plan (T37)	9.9%	(8.6, 11.2)	Transgender 39.7%; bisexual 24.7%
Attempted suicide (T38)	5.4%	(4.9, 5.8)	Transgender 17.3%; Hispanic/Latino 7.0% vs White 4.5%
Self-harm without suicidal intent (T34)	15.1%	(13.4, 16.8)	Transgender 53.2%; nonbinary 38.6%; female 19.6%
Poor mental health most/all past 30 days (T33)	25.4%	(23.8, 26.9)	Transgender 64.4%; nonbinary 58.6%
Trusted adult for serious problem (T39)	79.9%	(78.8, 81.0)	Hispanic/Latino 75.2%; transgender 69.5%; White 81.9%
Stress manageable most days (T32)	63.3%	(61.5, 65.1)	Transgender 29.6%; female 56.6%; male 72.7%
Belongs at school (T144)	74.2%	(72.7, 75.7)	Transgender 46.5%; nonbinary 56.6%
Feels safe at school (T154)	94.7%	(93.8, 95.6)	Transgender 78.6%
Treated badly due to race/ethnicity (T48)	4.6%	(4.0, 5.2)	Black/African American 18.8%; Hispanic/Latino 11.3%; White 1.4%
Went hungry due to lack of food (T128)	8.8%	(7.7, 9.9)	Hispanic/Latino 13.6%; transgender 14.6%

A county average of 25.2 percent on persistent sadness sits alongside a transgender rate of 66.4 percent. Both come from the same survey. Reporting only the average is a choice to not see the students most at risk. Any data system built on county-wide averages will consistently fail the students who need it most. This is why disaggregation by race, language, and gender identity is not a technical preference in this project. It is the foundation.

The Three Districts: Current Measurement Landscape

District Profiles

All three districts have CO-MTSS frameworks in place. Each employs counselors and mental health staff. Each is genuinely trying. The challenge is not absent infrastructure. It is that the data infrastructure each district has built does not connect to the other two and does not measure the behavioral health indicators that matter most for planning.

Table 2. District Profiles

Sources: Groves, Jorgensen, & Olsen (2024); CDE Unified Improvement Planning Dashboards (2025 to 2026); district websites.

	Estes Park (EPSD)	Poudre (PSD)	Thompson (TSD)
Enrollment (2025-26)	905 (down from 1,014 peak)	28,343	13,800 (down from 14,717 in 2021-22)
Counselor ratio	253:1	244:1	266:1
Hispanic/Latino	32.7%	20.2%	23.4%
Multilingual Learners	22.2% (4x higher than PSD or TSD)	4.8%	5.1%
Free/Reduced Lunch	39.7%	36%	37.8%
Graduation rate	84.3%	86.9%	90.9%
CDE accreditation 2025	Accredited (62.4%)	Accredited (67.4%)	Accredited (60.5%)
Academic achievement	Approaching (46.0%)	Meets (72.7%)	Approaching (60.7%)
Key assets	MTSS function; Cultural Liaisons; ELD program; Salud health partnership	27 MH specialists; bilingual support; Research & Evaluation dept.	Formal MTSS dept.; Comprehensive Screening system; eduCLIMBER platform; Sunrise Thompson Health Clinic

EPSD's Multilingual Learner rate of 22.2 percent is nearly four times higher than PSD or TSD, up substantially from 16 percent reported in the 2024 Tri-District Report. Its Free/Reduced Lunch rate of 39.7 percent is now the highest of the three districts. Both of these increases matter for how any shared data system needs to be designed for EPSD specifically.

PSD is the only district rating Meets on Academic Achievement every year from 2019 to 2025. Both PSD and TSD carry a Low Participation flag every year, meaning their assessment data is incomplete. EPSD's accountability data is largely suppressed because its enrollment of 905 means many subgroup cells fall below minimum reporting thresholds. A district that cannot be benchmarked externally, and cannot benchmark itself internally, is invisible in any regional planning conversation. Closing that gap is one of the core purposes of this project.

What the Districts Currently Measure

Five survey instruments are active across the three districts. The full item-level crosswalk across 29 construct categories is in the companion workbook.

Table 3. Active Survey Instruments

Instrument		District	Grades	Anon.	What it covers
Panorama SEL (two versions)		EPSD	K–12	Yes	Academic effort; emotional regulation; trusted adult; can be yourself around a trusted adult
CO Student Perception Survey		EPSD	Elem + HS	Yes	Teacher relationships; classroom belonging; cultural respect
PSD Student Connections Check-In		PSD	4–12	No	Adult connections; peer connections; food security; stress manageability; school safety; SEL across 5 domains
Panorama SEL Supports & Environments		TSD	6–12	Yes	Teacher relationships; school climate; belonging; safety
HKCS (2025 specific version)	TSD-	TSD (all)	HS	Yes	Mental health; self-harm; suicidality; racism; substance use

One distinction matters here: PSD’s Connections Survey is not anonymous. Every response is linked to the individual student and accessible to PSD staff. All other instruments are anonymous. This affects how openly students may respond on sensitive topics and must be factored into how PSD data is interpreted.

Where the Instruments Align

Three constructs appear across most instruments in forms that could support cross-district comparison with some coordination.

Trusted adult connections. EPSD’s Panorama, PSD’s Connections, and the HKCS all measure whether students have an adult at school they can count on for help. The 2023 county baseline is 79.9 percent (95% CI: 78.8, 81.0), with a meaningful gap between Hispanic/Latino students (75.2 percent) and White students (81.9 percent). TSD’s Panorama measures teacher respect but not the direct trust question. Three of four instruments cover this. TSD needs one item.

Stress manageability. PSD Connections Section D and HKCS Q21 use word-for-word identical phrasing: “My stress level is manageable most days.” This is the only item in the current ecosystem where two instruments already produce directly comparable data. EPSD and TSD do not collect it.

Belonging and safety. All four instruments measure belonging but with wording variations that prevent direct aggregation. PSD, TSD, and the HKCS measure school safety. EPSD has no safety item.

What No District Is Currently Measuring

A student who seriously considered suicide last year does not appear in any district annual dataset. The HKCS captures this biennially but cannot identify individual students for follow-up. That is not a small gap. It is the core failure of the current system.

The following constructs are measured only by the HKCS. No district currently collects annual school-level data on any of them.

- **Persistent sadness and hopelessness (HKCS T35):** 25.2% overall (95% CI: 23.0, 27.3). Transgender 66.4%. Hispanic/Latino 29.6%. The students this item captures are the quiet ones, not generating referrals, not appearing anywhere in the system.
- **Suicidal ideation, planning, and attempts (HKCS T36 to T38):** 11.9% seriously considered suicide; 9.9% made a plan; 5.4% attempted. Transgender: 50.0% ideation, 39.7% planned, 17.3% attempted. Hispanic/Latino attempt rate (7.0%) exceeds the White rate (4.5%).
- **Deliberate self-harm (HKCS T34):** 15.1% overall. Transgender 53.2%. Nonbinary 38.6%. Female 19.6%.
- **Poor mental health frequency (HKCS T33):** 25.4% reported poor mental health most or all of the past 30 days. Transgender 64.4%.
- **Experiences of racism (HKCS T48):** Black/African American 18.8%. Hispanic/Latino 11.3%. White 1.4%. This is the only item in the ecosystem that directly asks students about racism. Without it, the data system cannot surface the racial disparities it exists to address.

What Five Years of PSD Data Shows

PSD's Connections Survey is the most developed instrument in the ecosystem and the closest available example of what school-level behavioral health measurement can produce. The 2025 administration reached 14,766 students at a 79.3 percent response rate across grades 4 to 12, with the respondent pool closely matching the full student population across every demographic dimension (Poudre School District, 2025).

Two findings from five years of PSD data matter most for this project.

The first is about equity. Students receiving free meals scored 68.2 percent on the SEL composite in 2025 versus 77.4 percent for non-low-income peers, a gap of 9.2 points that has been present and persistent every year of the dataset. Hispanic students score 2.4 points below White students on Student-to-Adult connections. The Self-Awareness subscale, which measures cognitive emotion regulation, is the weakest SEL subscale at 67.1 percent. Its lowest item, "I know how to identify and change my negative thoughts," scored 62.3 percent. The students scoring low on these items are not generating referrals. They are the students Weist and colleagues (2018) describe as internalizing: struggling quietly, invisible to systems built around behavioral incidents.

The second is about what students are saying directly. When 2025 respondents were asked what they most wanted to learn about at school, 6,900 students selected mental health. That is the top response by a wide margin, more than 50 percent above the next categories. Students are not waiting to be asked. They are already telling the system what they need. The question is whether the data infrastructure is designed to hear it.

The SEL composite also dropped 11.2 points between 2023 and 2024, from 84.7 to 73.5 percent, partially recovering to 74.8 percent in 2025. It has not returned to pre-2024 levels. That kind of movement in a well-designed instrument reflects something real happening for students at scale. A data system with only one district measuring SEL cannot tell you whether that pattern is specific to PSD or is present across all three districts. Shared measurement makes that question answerable.

The equity gaps are only visible because the data is disaggregated. A PSD composite of 74.8 percent on SEL looks acceptable. The 9.2-point income gap inside that composite does not. The same principle applies to every shared metric this project recommends. Disaggregation is not an analytical option. It is what makes the data worth collecting.

Gap Summary

Table 4. Constructs Missing from District Annual Surveys

All rates verified from HKCS High School Region 2 Report, 2023, with 95% confidence intervals. Full crosswalk in companion workbook.

Severity	Construct	2023 Verified Rate	Missing from
Critical	Persistent sadness/hopelessness	25.2%; transgender 66.4%; Hispanic/Latino 29.6%	All three districts
Critical	Suicidal ideation	11.9%; transgender 50.0%	All three districts
Critical	Suicide planning and attempts	9.9% planned; 5.4% attempted	All three districts
Critical	Deliberate self-harm	15.1%; transgender 53.2%	All three districts
Critical	Experiences of racism	Hispanic/Latino 11.3%; Black/African American 18.8%	All three districts
High	Poor mental health frequency	25.4% most or all of past 30 days	All three districts
High	Direct trusted adult item	79.9%; Hispanic/Latino 75.2%	TSD only
High	School safety item	94.7%; transgender 78.6%	EPSD only
Medium	Help-seeking capacity	54.2% would tell a trusted adult	EPSD and TSD
Medium	Food security	8.8%; Hispanic/Latino 13.6%	EPSD and TSD

The data asymmetry across districts is itself a finding. PSD has five years of Connections trend data and a public Power BI dashboard. TSD has CDE accountability data and participates in the biennial HKCS. EPSD has neither public outcome data nor publicly reported CDE accountability ratings, because with 905 students many cells fall below minimum reporting thresholds. A district that cannot track its own progress over time, and cannot be compared to neighboring districts, is effectively invisible in any regional planning effort. Closing that asymmetry is one of the core purposes of this project.

What This Scan Establishes

This Environmental Scan has documented the current state across all three districts. It has shown what each district measures, where the instruments align, and where the critical gaps are. It has verified, from primary sources with confidence intervals, that the most important behavioral health indicators are absent from every district's annual measurement system. And it has shown, through five years of PSD data, that school-level measurement done well is both actionable and equity-sensitive.

The next deliverables build directly on this foundation. Deliverable 4 will identify specific shared metrics, starting from the HKCS baselines documented here. Deliverable 5 will evaluate universal screening tools that can find the students the current instruments are not designed to find.

The county has \$24.5 million annually committed to behavioral health. The HKCS provides free, verified baselines for every indicator that matters. PSD has five years of evidence that measuring student well-being at the school level produces information that a district can act on. All three districts have CO-MTSS frameworks ready to connect to a shared measurement system. What has been missing is the coordination to make all three districts part of the same picture. That is what this project is building.

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